

Medicare Part D Drug Analytics, 2019 to 2023

The macro picture in one screen. Live dashboard at [partd-analytics-hari.streamlit.app](#)

2023 Part D Total Spend

5-Year Growth

GLP-1 Diabetes Spend, 2023

Off-Label Ozempic Estimate, 2023

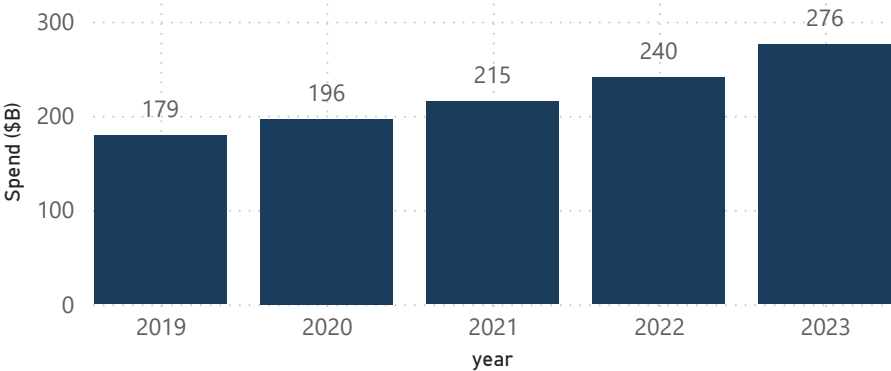
\$275.9

\$97.0

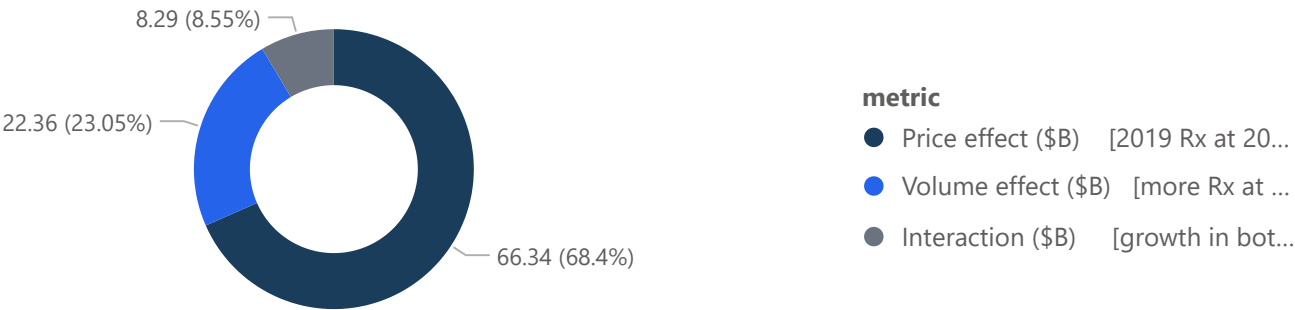
\$22.2

593

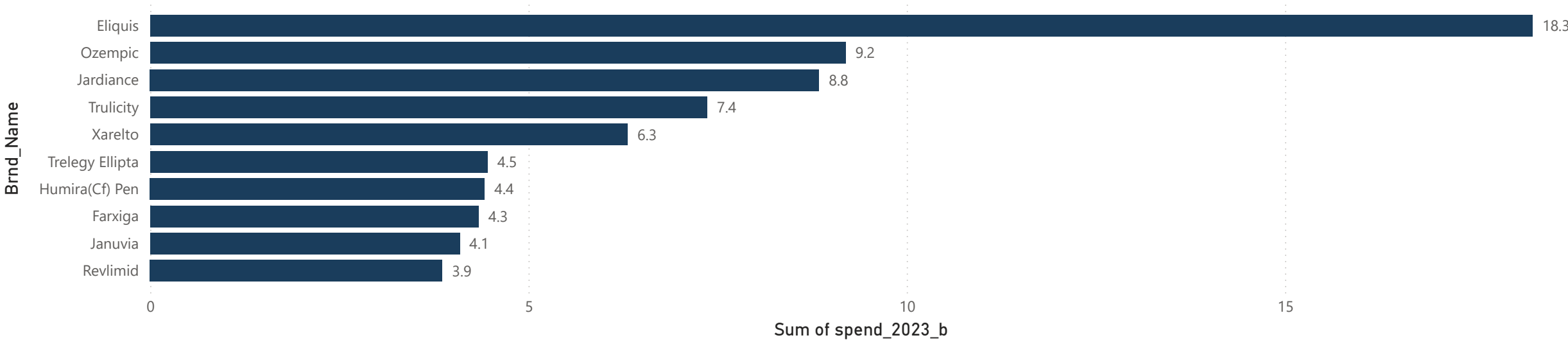
Annual Part D Total Spend (\$B)



What Drove the \$97B Growth



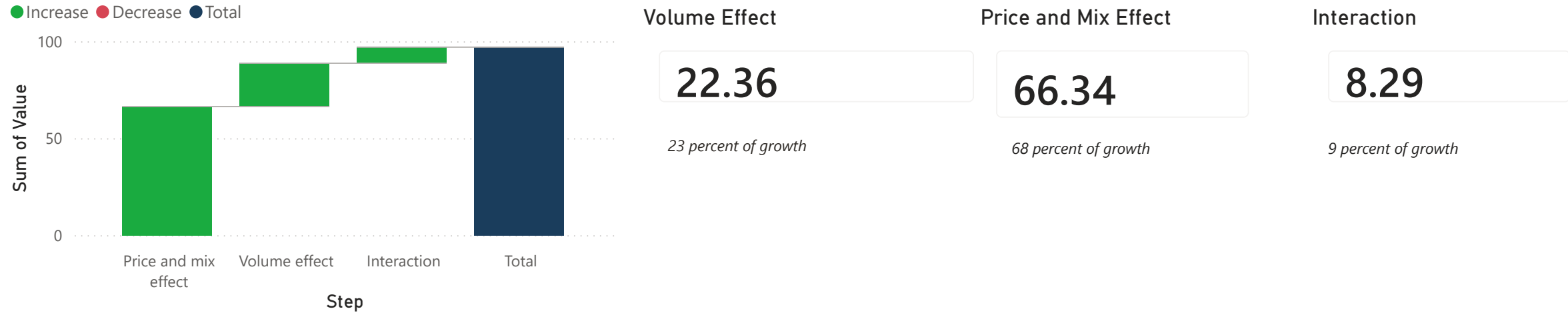
Top 10 Part D Drugs by 2023 Spend (\$B)



Where the \$97B in growth came from

Total spend growth from 2019 to 2023 splits algebraically into three components. Volume, the change in claims at the old price. Price and mix, the change in price at the old claims. Interaction, the cross-term. The three components close exactly to total growth.

Part D Growth Decomposition, 2019 to 2023 (\$B)



The insight. Three-quarters of Part D growth came from price and mix shift, not from more prescriptions being filled. This means the program's growth is structurally a pricing story, not a utilization story. Patients are migrating from older lower priced drugs to newer higher priced ones. Volume management has less leverage than price and formulary management.

Generic Dispensing Rate Optimization

Where brand spending sits on molecules that already have generic alternatives, sorted by absolute savings opportunity.

Brand Spend on Genericized Molecules (\$B)

Industry Benchmark GDR (%)

Realistic Annual Savings (\$B)

\$17.3

90

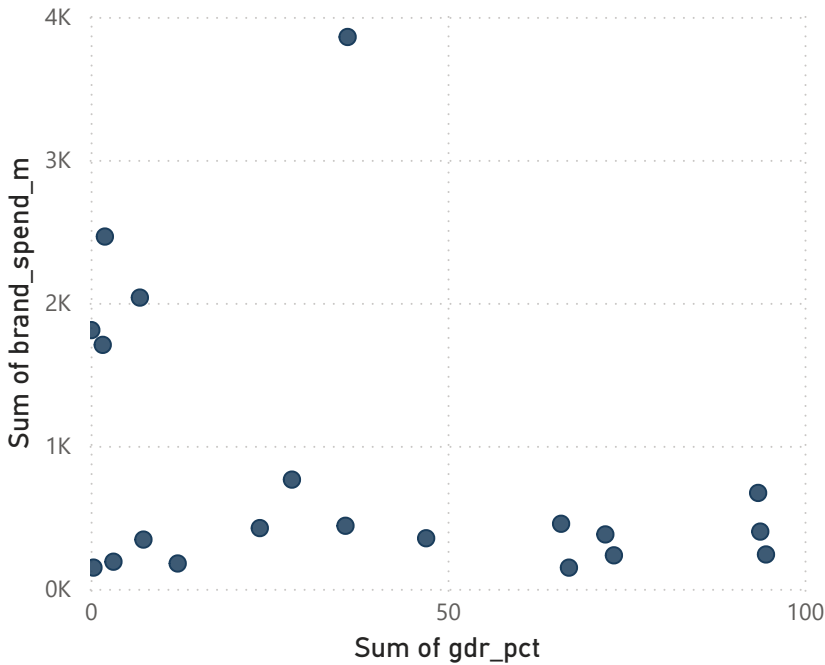
\$4.0

Top 20 molecules only

PBM standard target

\$3 to \$5B range at 90% target

GDR vs Brand Spend by Molecule, Top 20 (2023)



Top 20 GDR Optimization Opportunities

Molecule	Brand \$M	Generic \$M	GDR %
Lenalidomide	3,859.80	1,681.30	36.00
Aripiprazole	670.90	372.70	93.50
Metformin HCl	380.70	247.50	72.10
Lurasidone HCl	455.20	233.00	65.90
Icosapent Ethyl	764.00	228.90	28.20
Oxycodone HCl	400.50	151.90	93.80
Glatiramer Acetate	441.20	151.50	35.70
Divalproex Sodium	148.40	141.80	67.00
Teriflunomide	353.90	110.40	47.00
Dimethyl Fumarate	234.50	104.20	73.30
Teriparatide	425.00	89.10	23.70
Lamotrigine	240.50	70.90	94.60
Tiotropium Bromide	1,706.50	25.50	1.70
Venlafaxine HCl	177.40	22.70	12.20
Insulin Lispro	2,037.10	22.10	6.90
Bupropion HCl	344.60	20.80	7.40
Insulin Aspart	2,464.00	16.80	2.00
Pitavastatin Calcium	189.90	3.10	3.20
Nifedipine	148.60	0.80	0.40
Insulin Degludec	1,809.60	0.20	0.10

Insulin caveat. Four of the top five GDR opportunities by absolute spend are insulins. Their low GDR is not a clinical limitation, it is a rebate contracting one. UM intervention without contract renegotiation produces minimal savings. See the P&T memo for handling.

GLP-1 Receptor Agonists, the Class Reshaping Part D

From 2.9 percent to 8.1 percent of Part D in five years. Projected 17 to 21 percent by 2027.

GLP-1 2023 Spend (\$B)

5-Year Growth (x)

Off-Label Ozempic 2023 (K beneficiaries)

\$22.2

4.20

593

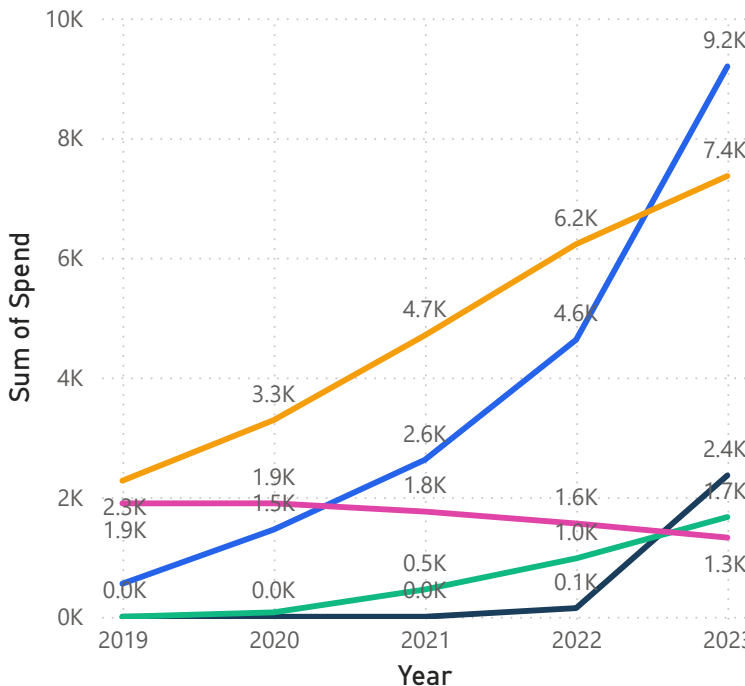
8.1 percent of all Part D

\$5.3B to \$22.2B

Estimated via beneficiary differential vs Trulicity

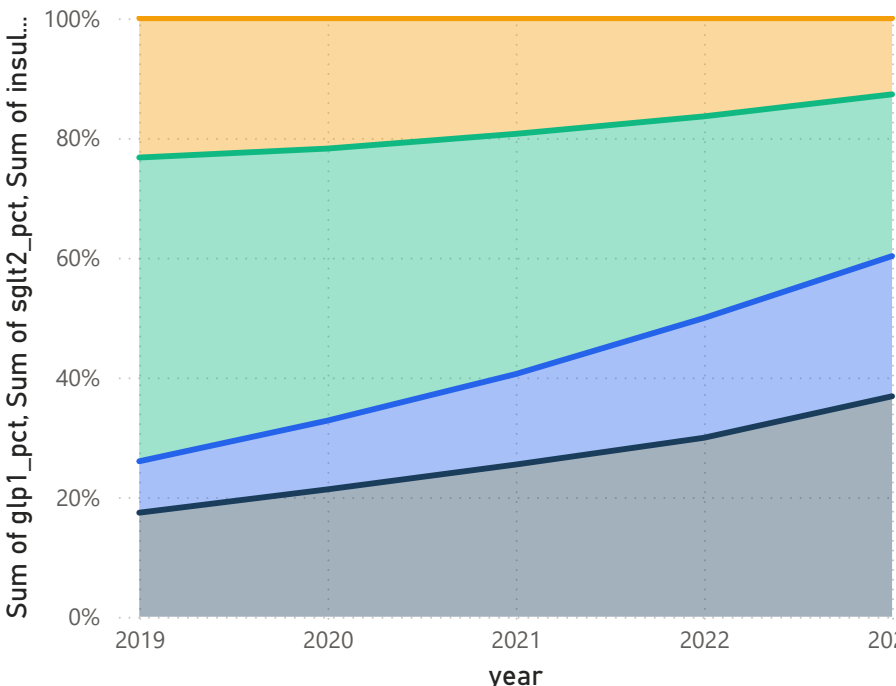
GLP-1 Spend by Brand, 2019 to 2023 (\$M)

Brand Mounjaro Ozempic Rybelsus Trulicity Victoza



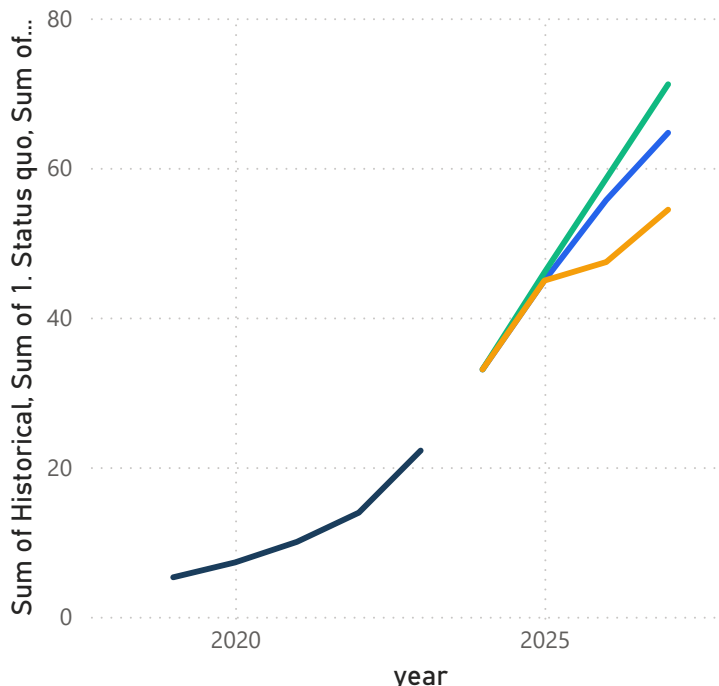
Diabetes Class Share Over Time (%)

Sum of glp1_pct Sum of sglt2_pct Sum of insulin_pct Sum of other_...



GLP-1 Spend Forecast, 2023 to 2027 (\$B)

Sum of Hist... Sum of 1. St... Sum of 2. ... Sum of 3. I...



The policy scissors finding. IRA Drug Price Negotiation savings on Ozempic alone, around \$10B per year by 2027, roughly equal hypothetical Medicare obesity coverage cost, around \$6.5B per year. Combining both policies could nearly double the GLP-1 treated patient population while netting Medicare savings. Neither policy alone produces that outcome.